Jefferson Health

June 13th, 2025

Dr. Mehmet Oz
Administrator
Center for Medicare and Medicaid Services
Office of the National Coordinator for Health Information Technology
Department of Health and Human Services

RE: [CMS-0042-NC] RIN 0938-AV68, Request for Information; Health Technology Ecosystem

Dear Administrator Oz,

Jefferson Health has been serving Pennsylvania and New Jersey communities as an academic safety net provider since 1825. Our 32 hospital locations provide care for more than 8.8 million hospital and outpatient visits each year. Jefferson Health Plans provides health insurance access for Medicare, Medicaid, and ACA beneficiaries.

Jefferson is committed to creating unparalleled value and reimaging health and education for our community. In that commitment, Jefferson is accountable for nearly 750,000 lives in value-based arrangements and an additional 350,000 lives in Jefferson Health plans. We share CMS's goal to improve the digital ecosystem by providing more high-quality value-based patient care models. Jefferson thanks CMS for the opportunity to respond to the value-based section of this RFI.

1. Digital Health Adoption
VB-1. What incentives could encourage APMs such as accountable care organizations (ACOs) or participants in Medicare Shared Savings Program (MSSP) to leverage digital health management and care navigation products more often and more effectively with their patients? What are the current obstacles preventing broader digital product adoption for patients in ACOs?

Care management activities are best provided in collaboration with patients' care teams and primary care. We recommend that care navigation products be embedded within the electronic medical record that the patient's primary care team is also using to document their care. This prevents inefficiencies arising from care managers documenting across multiple systems and the patient's care team not being able to see the full picture.

Care Management software, risk stratification and other tools are an additional significant expense to ACOs. A financial incentive or credit tied to adoption would help organizations be able to afford an investment in this technology. This can be especially important for new providers who do not have a history of shared savings payouts to justify the expense.

Programs like remote patient monitoring (RPM) that leverage technology to monitor patients in their home often suffer from low patient engagement due to patient cost sharing requirements. CMS has a reimbursement mechanism through codes (99453, 99454, 99457, 99458,98980, 98981) for RPM services. However, Jefferson finds patients have a barrier to engagement when cost sharing requirements are applied. Jefferson encourages CMS to reduce this administrative burden and increase patient adoption.

Regarding digital products, there are several barriers potentially limiting patients' adoption of digital health tools. These barriers generally fall into two domains: 1) technical readiness - which includes factors such as access to needed resources (devices and internet connectivity) and knowledge in use of technologies / platforms (digital literacy) and 2) quality-of care readiness – which includes factors such as patients' trust in the quality of care that they can receive via digital health and concerns about maintaining privacy of data with digital health tools. (Rising KL, Guth A, Gentsch AT, Gonzalez KM, Hass R, Shughart L, Gelfer A, McVane M, Worster B, Leader AE. Development and preliminary validation of the Digital Health Readiness Screener. JAMA Network Open. 2024;7(9):e2432718)

Implementation of standard screening for patients' digital health readiness in clinical practice settings and ensuring availability of dedicated resources to address identified barriers (e.g., credits to pay for internet connectivity when needed, community health workers to help address trust barriers) is likely to significantly increase patients' adoption of digital health products.

VB-2. How can key themes and technologies such as artificial intelligence, population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

Risk stratification, quality measurement, care coordination, and other core elements are foundational to successful population management and success in value-based contracts. These elements can often take years to fully implement and require additional funding. This often becomes a glidepath for ACOs who need to spread investments over multiple years as incremental success is achieved, and value-based payouts are realized.

We would recommend integration into APM requirements that match several levels of maturity. Several of these areas, such as patient engagement, require manual report creation. For these reasons, we would recommend an attestation process incorporated into APM requirements for various maturity levels.

Jefferson continues to utilize and explore where Artificial Intelligence (AI) can make care more efficient. AI can be leveraged well for patient flow or patient management. Patient flow, agentic agents, asynchronous chat bots and symptom checkers can risk stratify and enhance triage to the most efficient site of care; especially when combined with a telemedicine visit and good care coordination. AI can be leveraged to schedule appointments, perform virtual check-in, and

possible virtual discharge. It can replace self-check-in and ask more detailed questions in a purposeful way.

VB-3. What are essential health IT capabilities for value-based care arrangements?
a. Examples (not comprehensive) may include: care planning, patient event notification, data extraction/normalization, quality performance measurement, access to claims data, attribution and patient ID matching, remote device interoperability, or other patient empowerment tools.
b. What other health IT capabilities have proven valuable to succeeding in value-based care arrangements?

We consider essential health IT capabilities for value-based contracts to be:

- Patient Event Notification or ADTs including skilled nursing facilities.
- Data extraction/normalization for claims, pharmacy, and clinical data. In addition, the ability to pull quality metric data on outcomes such as A1c value is also critical to success with payers.
- Quality performance measurement including Electronic Medical Record (EMR) ability to send electronic quality metrics. Run rate analytics is especially important allowing clinicians to understand the number of patient care gaps they need to close to manage performance at the population level.
- Access to timely claims and pharmacy data. Pharmacy data, especially medication history and dispensed data, functions best as a weekly feed to promote medication adherence work and outreach in a timely fashion.
- Patient identification logic and methodology allowing for patient matching across technology platforms.
- Electronic outreach tools such as EMR communication and text messaging capabilities.
- Risk stratification tools to predict risk of readmission, future utilization, and other factors with health-related social needs incorporated.
- Customer relationship management tools for outreach to the population to close care gaps.

VB-4. What are the essential data types needed for successful participation in value based care arrangements?

- Clinicians in alternative payment models need medical claims, pharmacy claims, provider attribution, benchmarks, and disease burden capture or risk adjustment data to be successful in value-based care arrangements.
- Pharmacy medication history data provides medications dispensed, including drug name, dosage, quantity, date filled, prescribing physician, and refill history.
- Attribution data that distinguishes between primary care vs specialist attribution.
- Health related social needs data from screening and other public sources are important to risk stratify the population and provide the right care team members.

2. Compliance and Certification

VB-5. In your experience, how do current certification criteria and standards incorporated into the ONC Health IT Certification Program support value-based care delivery?

The ONC Health IT Certification Program provides important support for value-based care delivery. Standards such as: "Capability to provide clinical decision support", "Capacity to capture and query information relevant to health care quality", "Capacity to exchange electronic health information with, and integrate such information from other sources" are critical to help clinicians at the point of care with the patient. Information from outside data sources is especially critical for care managers who are organizing complex care for patients who may have multiple providers, medications, and interactions with the health system.

VB-6. What specific health information technology capabilities that could benefit APMs are not currently addressed by existing certification criteria and standards that should be included under the ONC Health IT Certification Program?

The ONC Health IT Certification Program provides a solid base of criteria, but there are many functions that would be useful as value-based care has evolved. Health systems continue to need the flexibility to integrate 3rd party technology solutions into their EMR.

- Risk stratification models to analyze populations and the ability to facilitate easier documentation for value-based FFS codes are two examples. Services, such as chronic care management (99490, 99439, 99487,99489,99491, G0506) community health integration (G0019, G0511), and advanced care planning (99497, 99498) are important for patient outcomes but often are not implemented due to technology challenges and onerous documentation standards.

- Many EMRs do not have adequate risk stratification tools to help clinicians and care managers identify patients and get them to the right care programs. Risk stratification models often lack claims information as well as health related social needs variables to improve accuracy.
- Integration with Community Resource Directories.
- Data feedback mechanism indicating a community-based organization accepted and acted upon a referral.
- Longitudinal Care Planning with Shared Goal Setting - certification for health IT that supports dynamic, shared care plans involving the patient, multiple providers, and even family caregivers, with clear goal setting and progress tracking.
- Patient-Generated Health Data (PGHD) Integration and Analysis

VB-7. How can technology requirements for APMs, established through CEHRT or other pathways, reduce complexity while preserving necessary flexibility?

CMS can reduce complexity for IT and analytics teams by taking an API-first approach. Jefferson encourages CMS to strongly emphasize standardized APIs (like FHIR) for data exchange. This will allow us to innovate and scale more efficiently. Continued development and expansion of the Minimum Viable Data Sets (like USCDI) will also reduce complexity.
Regulatory reporting leveraging solely EMR data via EcQMs is a complex and antiquated solution. Allowing ACO's to report using registry data which blends clinical and administrative data or limiting reporting to the Medicare population would reduce administrative lift and complexity.

VB-8. How can other HHS policies supplement CEHRT requirements to better optimize the use of digital health products in APMs? As an example, requirements under the Conditions of Participation for hospitals (42 CFR 482.24(d)) require hospitals to transmit electronic patient event notifications to community providers. What barriers are in place preventing APM participants from receiving the same notifications?

Many APM providers do receive ADT notifications from community providers today. This is often delivered via health information exchanges. The "last mile" cost of participating in a health information exchange can often be a barrier for some types of providers like skilled nursing facilities and smaller primary care offices. Notifications need to be ingested into a workflow where a care manager, clinician, or other team member can act on the alert.

VB-9. What technology requirements should be different for APM organizations when comparing to non-APM organizations (for example, quality reporting, and interoperability)?

APM entities need to be able to ingest attribution rosters, share quality metric information, and configure workflows in their electronic medical record (EMR) at a minimum. As financial exposure increases to the provider/system, APMs must be able to benchmark their claims data performance, support care management, social work and pharmacy workflows, and the ability to measure more complex metrics such as medication adherence in a timely fashion.

VB-11. What specific interoperability challenges have you encountered in implementing value-based care programs.

Jefferson participates in value-based contracts across multiple payers in all lines of business. We often experience different claim file types and standards across these payers that all require unique loads and maintenance to keep operational. Each EMR has a unique build and workflow required for value-based operations. Lack of standardization requires additional technology support and administration to work with independent physicians.

Many payers are utilizing "black box" risk adjustment methodology that cannot be replicated by the organization, making it difficult to predict and measure performance and financial outcomes in value-based contracts. Health information exchanges provide a vital source of information to track populations, but many community providers do not participate in them, making interoperability difficult.

Data flow and updates continue to be a challenge across providers and payers. Care managers and other team members often find out that a patient's primary care physician or contact information has been changed. There is not an easy, interoperable way to update this with their payer or other care team providers who may sit outside our EMR.

VB-12. What technology standardization would preserve program-specific flexibility while promoting innovation in APM technology implementation?

- Da Vinci Clinical Data Exchange (CDex)
- Digital Quality Measurement specification expansion and supporting FHIR extensions and profiles.
- Gravity Project expansion to include Comprehensive Health Related Social Needs focusing on information on community resource referrals and their outcomes.
- Universally standardized FHIR extensions for the input features of advanced risk assessment predictive models.

- VBC needs broader population-level reporting standards on utilization, cost, and overall financial performance at varying levels: NPI, Practice, Department, TIN, ACO, and Contract.
- Care Coordination standards to track the full lifecycle of activities including referrals, acceptance, completion, and outcome, across different EMRs and Payer systems.
- National directory of FHIR endpoints.

VB-14. How could implementing digital identity credentials improve value-based care delivery and outcomes?

Implementing a robust digital identity credentialing framework can significantly improve value-based care by enhancing patient engagement and control over their health data. Patients will be able to easily gather their health information from various payer and provider portals, regardless of the vendor. This also empowers them with granular control over data sharing through specific consent options and transparent audit trails, while simultaneously improving patient matching across systems, which cuts down on medical errors and inefficiencies.

For providers, digital identity credentials will boost efficiency and collaboration. Clinicians will benefit from secure Single Sign-On (SSO) access to multiple EMRs and applications, enabling secure, context-aware access to patient data based on their role and needs. This streamlines referrals and care transitions with verified patient and provider identities, fostering seamless, multidisciplinary team-based care coordination.

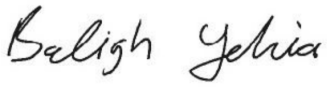
Digitizing identities will also improve numerous administrative processes. It will eliminate the need for costly roster management and de-duplication efforts, accelerate patient registration and check-in, and reduce duplicative paperwork and unnecessary tests. Furthermore, it will expedite provider credentialing and onboarding within VBC networks, leading to more efficient operations.

Ultimately, these improvements will directly impact VBC outcomes. Better data access and care coordination will improve performance on contracts and reduce overall healthcare costs by minimizing administrative burden, preventing duplicative services, and reducing fraud. Patient safety and engagement will be enhanced through accurate and timely access to health data, and VBC models will benefit from more accurate risk adjustment. This foundational identity layer will also accelerate innovation, paving the way for new VBC applications.

VB-15. How could a nationwide provider directory of FHIR endpoints help improve access to patient data and understanding of claims data sources? What key data elements would be necessary in a nationwide FHIR endpoints directory to maximize its effectiveness?

A "digital yellow pages" is crucial for innovation by VBC organizations. This would allow for faster development of our data integration, analytics, clinical decision support, disease burden capture, and care coordination programs. It would also increase patient engagement and experience. Currently, VBC organizations are beholden to their population health or EMR vendors which monetize and gate access to this information. This would open opportunities for new open-source solutions to develop in the marketplace.

Sincerely,



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